Improving Lives

Home Health Referral

Fax (702)899-4479

Avery Home Health
5660 W Flamingo Rd Ste A
Las Vegas, NV 89103
Tel#: (702) 416-0611
Email: admin@averyhomehealth.net

Patient Name: Gertrude Dixon **DOB:** 02.16.1951

skip if demographic sheet is attached

Address: 440 Elysian Plains Ct. Las Vegas, NV 89145

Tel #: 702.271.2324 Emergency Contact #:

1st Insurance & Policy #: Medicare ; 8DX2KM6MK85

2nd Insurance & Policy #:

Medical Condition related to Home Health

- ☐ CVA
- ☐ AMI
- ☐ Diabetes
- ☐ COPD
- ☐ Cardiac Surgery
- ☐ ESRD
- ☐ Heart Failure
- ☑ Joint Replacement
- ☐ HTN
- ☐ PNA
- ☐ Dementia/ALZ/ Parkinson's

Other, please specify:

a. left total knee
b. arthroplasty
c.
d.
e.
f.
g.
h.
i.
j.

Homebound due to:

- __ Unable to ambulate
- __ Requires assistive device, wheelchair, walker
- __ Requires assistance of another person to leave home
- __ Requires use of special transportation
- ✓ Post operation
- __ Confusion/Cognitive limitations make it unsafe for patient to leave home
- __ Pain interferes with outside mobility
- __ SOB with exertion/activity, requires rest periods
- __ Recent hospitalization

Clinical findings or services required for patient's home health eligibility:

☐ Skilled Nursing

- __ Health education on disease process
- __ Medication management
- __ DM management
- __ Catheter/Drain care
- ✓ Post operative care
- __ Wound Care and pt/family training
- __ Ostomy care and pt/family training
- __ O2 Therapy/Trache management
- __ Others:

☑ Physical Therapy ☐ Occupational Therapy

- __ Gait/balance/transfer training
- __ Home safety /fall precaution
- __ Active ROM Exercises
- __ Post op exercises
- __ Education for safe use of assistive device
- __ Strengthening & endurance exercises
- __ Therapy for Pain Management
- __ ADLs/IADLs training
- __ Others:

☐ Speech Language Pathology Skilled evaluation/treatment:

- __ Aphasia
- __ Apraxia
- __ Dysarthria
- __ Dysphagia

☐ Medical Social Worker

- __ Psychosocial assessment
- __ assistance in application for community/government resources
- __ Others:

Ordering Provider: Ronald W. Hillock
Signature:
Phone #: 702.258.5508
Date: 03.09.2026

NORTHWEST LAS VEGAS
7455 W. Washington Ave Suite 160
Las Vegas, Nevada 89128

Phone: (702) 878-0393
www.nevadaorthopedic.com

HENDERSON
1505 Wigwam Pkwy Suite 330
Henderson, Nevada 89074

Patient Information

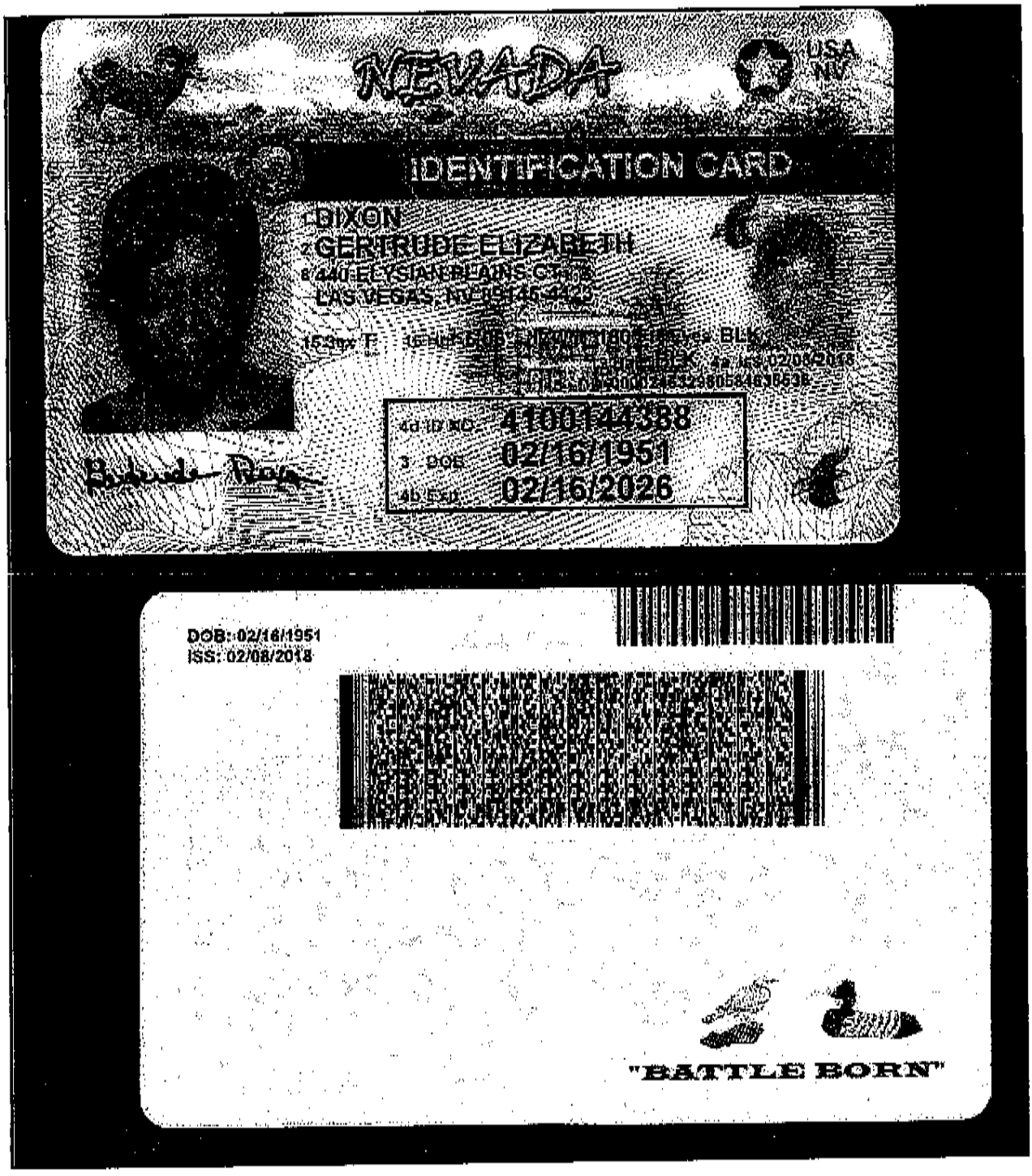
Patient Name:	**Gertrude E. Dixon**	**MRN#:**	296834	**Date of Birth:**	02/16/1951
Social Security #:	055-44-7050	**Language:**	English	**Referred by:**	RONALD W. HILLOCK, MD
Gender:	Female	**Race:**	Black or African American	**PCP:**	,
Marital Status:	Undefined	**Ethnicity:**	Not Hispanic or Latino		
Home Address:	440 Elysian Plains Ct Las Vegas, NV 89145-4423 Phone: (702) 242-3997 Mobile: (702) 271-2324 Email: vanurse440@aol.com	**Work Address:**	None	**Employer:** **Occupation:** **Employment Status:**	UNKNOWN None Retired
Guarantor Name:	Gertrude E. Dixon	**Relation to Guarantor:**	Self	**Guarantor Information:**	440 Elysian Plains Ct Las Vegas, NV 89145-4423 Phone: (702) 242-3997 Email: vanurse440@aol.com

Insurance Information

Carrier Information	Plan Information	Subscriber Information
MEDICARE PO Box 6776 Fargo, ND 58108-6776 Phone: (855) 609-9960 Plan Type: Undefined	Group #: NONE Plan #: 8DX2KM6MK85 Enrollment date: 02/01/2017	Gertrude E. Dixon DOB: 02/16/1951 440 Elysian Plains Ct Las Vegas, NV 89145-4423 (702) 242-3997
BCBS PO Box 5747 Denver, CO 80217-5747 Phone: (877) 833-5742 Plan Type: Undefined	Group #: 111 Plan #: R59725396 Enrollment date: 01/08/2017	Gertrude E. Dixon DOB: 02/16/1951 440 Elysian Plains Ct Las Vegas, NV 89145-4423 (702) 242-3997

Emergency Contacts

Michael V Dixon, Spouse(702) 279-1735


NEVADA
USA
NV
IDENTIFICATION CARD
1 DIXON
2 GERTRUDE ELIZABETH
4d ID NO. 4100144388
3 DOB 02/16/1951
4b EXP. 02/16/2026
DOB: 02/16/1951
ISS: 02/08/2018
"BATTLE BORN"

MEDICARE HEALTH INSURANCE

Name/Nombre

GERTRUDE E DIXON

Medicare Number/Número de Medicare

8DX2-KM6-MK85

Entitled to/Con derecho a	Coverage starts/Cobertura empieza
HOSPITAL (PART A)	05-01-2016
MEDICAL (PART B)	02-01-2017

You may be asked to show this card when you get health care services. Only give your personal Medicare information to health care providers, your insurers, or people you trust who work with Medicare on your behalf. **WARNING:** Intentionally misusing this card may be considered fraud and/or other violation of federal law and is punishable by law.

Es posible que le pidan que muestre esta tarjeta cuando reciba servicios de cuidado médico. Solamente dé su información personal de Medicare a los proveedores de salud, sus aseguradores o personas de su confianza que trabajan con Medicare en su nombre. **¡ADVERTENCIA!** El mal uso intencional de esta tarjeta puede ser considerado como fraude y/u otra violación de la ley federal y es sancionada por la ley.

1-800-MEDICARE (1-800-633-4227 / TTY: 1-877-486-2048); **Medicare.gov**

7455 W. Washington Ave Suite 160
Las Vegas, Nevada 89128

Phone: (702) 878-0393
www.nevadaorthopedic.com

1505 Wigwam Pkwy Suite 330
Henderson, Nevada 89074

Gertrude E. Dixon **MRN#:** 296834 **DOB:** 02/16/1951, 75 years

Date: 02/26/2026

Orthopedic Evaluation

History of Present Illness

The patient is a 75 year old female who presents for a pre-operative visit.The planned procedure is a LEFT TOTAL KNEE ARTHROPLASTY 3/11/2026 scheduled .

Patient presents to clinic today for preoperative evaluation in anticipation of left total knee arthroplasty. She has insidious onset of knee pain. She cannot take stairs due to the pain. She cannot walk more than 200 m at a time due to knee pain. She has attempted nonsteroidal anti-inflammatories without success. She has attempted a therapeutic exercise program without success.

Examination:

Head, ears, eyes, nose, and throat: Atraumatic and normocephalic.

Alert and oriented to person, place, time, and situation.

Skin: No open lesions, rashes or wounds. No café-au-lait spots. No dark or hairy patches present.

Vascular: Normal distal pulses all limbs. Capillary refill less than 2 seconds all limbs. No adenopathy present. No stasis changes about ankles. Normal finger and toenails.

Neurologic: Cranial nerves II - XII grossly intact. Light touch intact all limbs. Motor exam is 5/5 upper and lower extremities. Gait pattern is antalgic toward the left bearing full weight. Not using a cane, crutch or walker.

Hip: Limb lengths are symmetric. Nontender over the greater trochanter. No flexion contracture present. Nontender about either buttock, thigh or knee to palpation. Nontender about the lumbar spine. Negative straight leg raise.

Left knee examination shows that there is a 3 degree varus deformity. There is a slight effusion. There is patellofemoral crepitus.

Radiographs of the left knee were obtained and reviewed with the patient varus tricompartmental osteoarthritis. Bone-on-bone in the medial compartment. Large osteophytes present. 4 of 4 changes present.

Impression:

Left knee osteoarthritis, varus, chronic, severe. The patient has exhausted the nonoperative measures available for pain control. Pain is interfering with activities of daily living.

Recommendation and medical decision making:

Preoperative total knee counseling note

The patient has been counseled on the indications risks and benefits of total knee arthroplasty. After considering the options available to patient wishes to proceed with the surgery as discussed.

Specific risks discussed included but were not limited to: Death, bleeding, for blood transfusion, possible deep vein thrombosis, possible pulmonary embolus, possible infection, possible failure of implanted device or devices, possilbe dislocation, possible injury to unintended structures to include nerves, tendons, ligaments, blood vessels and muscles.

The patient was advised that steps will be taken to minimize these risks and it is impossible to eliminate all risk. Detailed conversation on specific risks and occurred. Treatment protocol for an infected total joint was discussed in detail. Specifically the patient was advised showed the joint become infected additional surgery and prolonged medications would be required. Additionally the patient was advised that in some extreme cases amputation is required to control infections. The patient was advised that prophylactic antibiotics would be given, and every step known to reduce the risk of infection would be employed in the course of this treatment plan. The patient was also advised on the protocol for reducing the risk of blood clot formation. The importance of continued pharmacologic treatment to reduce the risk of blood clot formation after leaving the hospital was also discussed in detail.

The patient was advised on the recovery timeline and rehabilitation protocols to follow surgery. They were advised that the typical inpatient hospital stay for a total knee replacement is 3-5 days. They were advised on the goals for discharge from the hospital. They were advised on the options available after discharge from the hospital. They were advised that inpatient treatment may need to be continued after discharge in a rehabilitation facility or skilled nursing facility. They were advised that our goal would be for discharged to home with visiting nursing and visiting physical therapy. Once they were mobile and able to travel in the community then therapy will to be continued on an outpatient basis. The first followup appointment after surgery typically occurs on postoperative day #14.

After considering the options available and having been given the opportunity to ask questions the patient appeared to understand the plan and my recommendations. They were willing to proceed with the surgery as discussed and were willing to consent for blood transfusion if necessary.

The patient was given a admission packet for the hospital. In the packet included an H&P as well as admission orders and instructions to the patient for the day of surgery. There were advised to consume no liquids or food by mouth after midnight the night before the surgery. Medications which could be taken before surgery were advised as well. There were counseled to discontinue all blood thinners for a minimum of 5 days before surgery.

Reviewed HPI, History and ROS at today's visit.

The dictated information, contained herein, was transcribed with voice recognition software. Although reviewed, the content may contain errors and/or omissions.

Patient History

Allergies

aspirin []: Nausea, Vomiting
Bayer Aspirin *ANALGESICS - NonNarcotic* []
Allergies Reconciled

Medications

Soma (350MG tablet, Oral) Active.
Gabapentin (300MG capsule, Oral) Active.
Furosemide (40MG tablet, Oral) Active.
Atorvastatin Calcium (40MG tablet, Oral) Active.
CeleBREX (200MG capsule, Oral) Active.
Levothyroxine Sodium (25MCG tablet, Oral) Active.
Potassium Chloride ER (10MEQ capsule, extend, Oral) Active.
Prempro (0.3-1.5MG tablet, Oral) Active.
Medications Reconciled.

Past Surgical

Abdominal Surgery Phreesia 07/28/2025
Hand Surgery
Other Surgery
Gallbladder Surgery Phreesia 07/28/2025
Knee surgery
Tubal Ligation Phreesia 07/28/2025
Nevada Orthopedic & Spine Center Consent for Procedure In Accordance with Nevada Revised Statute 41A.100
Patient # & Name: #@9104 @930Pat

Social

Disability Status: Not Disabled Phreesia 07/28/2025
Disabled no
Current work status 08/13/2013: yes
Hand Dominance: Right Handed Phreesia 07/28/2025
Exercise 08/13/2013: daily
Assistance Status: Home Assistance Available Phreesia 07/28/2025
Former smoker
Surgical Blood Transfusion: Approves surgical blood transfusion Phreesia 07/28/2025
Contract: Active Pain Contract Phreesia 07/28/2025
Exercise Tolerance Stairs 08/13/2013: 5+
Current Drug Rehab 08/13/2013: no
Drug Assessment: No Illicit Drug Use Phreesia 07/28/2025
Alcohol use 08/13/2013: occasional
Tobacco use 08/13/2013: former smoker; no cans of smokeless/chewing tobacco
Illicit drug use 08/13/2013: no
Home assistance available 08/13/2013: yes
Right or Left Handed right
Past Drug Rehab 08/13/2013: no

Other Past Medical

C-Pap Phreesia 07/28/2025
Therapy
Thyroid Disease
Pneumonia Phreesia 07/28/2025
Muscle pain
Shortness of Breath Phreesia 07/28/2025
Alcohol Abuse 08/13/2013: no
Respiratory Condition Phreesia 07/28/2025
Hypercholesterolemia
Patient Has Cardiologist Phreesia 07/28/2025
Hiatal Hernia
Hyperlipidemia Phreesia 07/28/2025
MRI
Hypertension Phreesia 07/28/2025
Injection(s)
Peripheral Vascular Disease Phreesia 07/28/2025
Gastroesophageal Reflux Disease
Kidney Disease Phreesia 07/28/2025
Asthma
Back Injury/Pain Phreesia 07/28/2025
Arthritis
Other Spine Problems Phreesia 07/28/2025
Fracture Treatment no
Anxiety Phreesia 07/28/2025
Depression Phreesia 07/28/2025
Diabetes Phreesia 07/28/2025
Thyroid Problems Phreesia 07/28/2025
Anemia Phreesia 07/28/2025

Problem List

Arthritis (osteo, primary), Knee - BIL
Problems Reconciled

Vitals

02/26/2026 11:49 AM
Weight: 206 lb **Height:** 65 in
Body Mass Index: 34.28 kg/m²
Pain Level: 8/10

"BMI >25: Losing Weight" handout given to address weight-loss options and benefits.

Review of Systems

General Not Present- Chills and Fever.
Gastrointestinal Not Present- Diarrhea and Nausea.
Musculoskeletal Not Present- Atrophy.
Neurological Not Present- Loss of Consciousness.

Assessment and Plan

Arthritis (osteo, primary), Knee - LT (M17.12)

Orders
Diet Education
Knee X-Ray, 3 Views (Taken and Read) - LT
PT Eval & Treat: Knee Physical Therapy Protocol

Additional Instructions
Dignity Health Physical Therapy

Follow up for post-op

Patient Education
Handout - BMI >25: Losing Weight

Electronically signed by: RONALD W. HILLOCK MD
on 02/27/2026